



The Social Implications of Advanced Healthcare Technology

- Many social scientists believe that technology not only is influenced by cultural values but also in return has a powerful and deterministic effect on culture and social structure — **technological determinism** theory.
- Society controls the introduction of new technologies (ex. computer, microwave and airbag)– **utopian view**
- Failure to systematically assess whether or not technology should be pursued – **dystopian view**
- Key advancements: *Cardiac technology, Critical care medicine, Medical imaging, Health care computers, Genomic medicine.*



Social implications:

- Advanced health care technologies create options for people and society... ICU, bypass surgery or assisted procreation
- They alter human relationships...time for discussion and decision
- Affect the entire health care system...health care inflation...technology assessment and cost-benefit analysis (e.g. organ transplant vs. prenatal screening)
- Stimulate value clarification thinking... value questions
- Create social policy questions...



Medical technology should be assessed on these five levels:

1. ***Biologic plausibility***: current understanding of the biology and pathology to support the technology
2. ***Technical feasibility***: safe and reliable delivery of the technology to the target patients
3. ***Intermediate outcomes***: immediate and specific effects of the technology
4. ***Patient outcomes***: overall and ultimate outcomes for the health of the patient
5. ***Societal outcomes***: external effects of the technology on society, including the ethical consideration.

Physician-Assisted Suicide

Arguments favoring it:

- Create peaceful and comfortable environment
- People have right to self determination
- Law could have some safeguards
- Public opinion poll favors (half for themselves, in case of USA)





Arguments opposing it:

- The physicians' duties and responsibility (inconsistent)
- Worried and depressed (might be treated)
- Interfere with patient-physician relationship (societal issues of terminally illness)
- Legalizing = likelihood of suicide being acceptable
- Progress had been made in dealing with last-stage pain (ex. palliative care)

Advance Directives and Navajo Culture



- (1) Determining if the patient is or is not willing to discuss negative information;
- (2) Preparing the patient by building rapport and trust, involving the family, giving an advance warning that bad news is coming, and involving traditional Navajo healers in the encounter
- (3) Communicating in a kind, caring manner that is respectful of traditional beliefs, for example, referring to the patient in third-party language rather than directly
- (4) Following through in a manner that fosters reasonable hope



Organ donation and transplantation

- Started in 1964 with kidney transplant...bone marrow, liver, pancreas and heart etc.
Positives clubbed with social and policy issues:
- Do the medical and quality-of-life outcomes for organ recipients (and donors) justify organ transplantation procedures? Sufficient therapeutic values! If yes then:
- Demand— supply— selection (complex formula or democratic principle)
- How can the supply of organs be increased? (increasing motivation keeping voluntary nature in mind)
- How much money should be allocated to organ transplant procedures?
- Who should (can?) pay for organ transplant procedures?



Three essential concerns of organ donation (according to Renee Fox and Judith Swazey):

1. “Uncertainty” about the therapeutic value of organ transplantation...rejection and long term problems
 2. Little consideration of the psychosocial dimensions of organ donation and receipt...commodification of body vs. gift of life
 3. Distributive justice vs. public good
- The unspoken yet powerful **social norms**... obligation vs. altruistic
 - Also receiving of transplants (***tyranny of the gift***)